

Discharge Summary Respiratory Medicine

Patient Demography Details :

Name : Rajinder Nath Sharma	Patient ID : MHLI.644284	IP No. : 269862
DOB : 22 OCT, 1944	Age/Gender : 81 Years/MALE	Primary Consultant : Pulmonology Deepak Bhasin Team
DOA : 24 MAY, 2026 14:09	Ward : MHLI-3RDFLR-MSP2	
Address : PUNJAB, Industrial Area Phase 6, PUNJAB, 16005	Mobile No. : 7018501966	

Date and Time of Discharge: 02 JUN, 2026 11:16

Final Diagnosis:

Diagnosis

Diffuse Alveolar Hemorrhage (DAH) presenting with hemoptysis
Chronic Kidney Disease with acute worsening of renal function
Coronary Artery Disease, status post CABG (2016)
History of Cerebrovascular Accident (2019)
Long-term oral anticoagulation prior to admission
ANA positivity with negative ENA profile
Sub-nephrotic range proteinuria
Presenting Complaints
Hemoptysis
Abnormal HRCT chest showing bilateral lung infiltrates
Worsening renal function

Presenting Complaints:

Hemoptysis
Abnormal HRCT chest showing bilateral lung infiltrates
Worsening renal function

History of Present Illness:

Mr. Rajinder Nath Sharma, a known case of coronary artery disease status post CABG in 2016, cerebrovascular accident in 2019, chronic kidney disease, and on long-term oral anticoagulation, presented with significant hemoptysis. Initial radiological evaluation demonstrated bilateral perihilar and basal ground-glass opacities, raising concern for diffuse alveolar hemorrhage. In view of the combination of pulmonary infiltrates, hemoptysis, and worsening renal function, he was admitted for further evaluation and management under the pulmonology team.

For post-hospital care at home, call Max@Home at 8744 888 888 (24x7 helpline). Services include Critical Care@home, Nursing Healthcare Attendant, Physio-therapy, X-Ray, Sample Collection, Medicine Delivery, Medical Equipment and more.

IN CASE OF ANY EMERGENCY PLEASE CONTACT 0172-5212000.

"This is a computer generated document and does not require any signature"

Past Medical History:

Coronary Artery Disease
CABG in 2016
Cerebrovascular Accident in 2019
Chronic Kidney Disease
Long-term oral anticoagulation therapy
Medication History
Oral anticoagulation therapy prior to admission
Other cardiac medications for CAD/CABG (as per previous prescriptions)

Medication History

eliquis
ecosprin
avas
betaloc
telm

On Examination:

Hemodynamically stable
Significant hemoptysis
Bilateral pulmonary infiltrates on imaging
Creatinine approximately 2.5 mg/dL on admission
Concern for pulmonary-renal syndrome

Couse in Hospital:

The patient presented with complaints of breathlessness for 3–4 days, hemoptysis for 2 days, and fever for 1 day. He also had a history of excessive sputum production for the preceding 2–3 months. Family members reported progressive forgetfulness and irrelevant speech over the last several months, for which he was undergoing evaluation and was noted to have significant carotid artery disease.

He was initially admitted under the Cardiology service for evaluation. HRCT chest demonstrated bilateral basal ground-glass opacities with relative subpleural sparing, raising suspicion for diffuse alveolar hemorrhage. In view of persistent hemoptysis and the possibility of pulmonary-renal involvement, he was shifted to the Medical ICU for further management.

At ICU admission, he was conscious, alert, mildly tachypneic, hemodynamically stable, and maintaining oxygen saturation adequately on room air. Considering the differential diagnoses of anticoagulant-related alveolar hemorrhage, infection, and autoimmune pulmonary hemorrhage, anticoagulation (Apixaban) and antiplatelet therapy were withheld. Empirical broad-

For post-hospital care at home, call Max@Home at 8744 888 888 (24x7 helpline). Services include Critical Care@home, Nursing Healthcare Attendant, Physio-therapy, X-Ray, Sample Collection, Medicine Delivery, Medical Equipment and more.

IN CASE OF ANY EMERGENCY PLEASE CONTACT 0172-5212000.

"This is a computer generated document and does not require any signature"

spectrum antibiotics were initiated. Autoimmune workup including ANA, ANCA, vasculitis profile, anti-GBM antibodies, urine examination, and cultures was sent.

During hospitalization, renal dysfunction became increasingly evident. Review of previous records revealed serum creatinine around 1.5 mg/dL in July 2025 with eGFR approximately 42 mL/min/1.73 m², confirming underlying CKD. Current creatinine increased to approximately 2.6 mg/dL with elevated blood urea levels, raising concern for acute kidney injury superimposed on CKD. Nephrology consultation was obtained from Dr. Manish Kumar Singla. Urine albumin-creatinine ratio was markedly elevated (1763–1824 mg/g), suggesting significant proteinuria and possible renal involvement as part of a pulmonary-renal syndrome. Telmisartan was withheld, nephrotoxic medications were avoided, and strict intake-output monitoring was advised.

Autoimmune evaluation showed ANA positivity (2+ coarse speckled pattern, endpoint titre 1:320), raising suspicion for an underlying autoimmune process. ANCA testing was negative. Vasculitis LIA panel was negative. Anti-GBM antibody testing subsequently returned negative. Because of ongoing hemoptysis and suspicion of autoimmune diffuse alveolar hemorrhage, intravenous methylprednisolone (Solu-Medrol 40 mg twice daily) was initiated.

Extensive infectious workup remained negative. Respiratory BioFire panel was negative. Sputum fungal smear was negative. Sputum and urine cultures showed no growth. Lower respiratory pathogen panel remained negative. Despite persistent small-volume hemoptysis during the initial hospital course, the patient remained hemodynamically stable without oxygen requirement and did not develop respiratory failure.

Blood pressure remained mildly elevated during admission, for which Cilacar was added. Serial monitoring showed stable respiratory status, adequate urine output, and no progressive renal deterioration. Hemoptysis gradually decreased in frequency and subsequently resolved. Throughout the admission, the patient remained conscious, oriented, afebrile, and maintained oxygenation on room air.

A repeat HRCT Chest performed on 02/06/2026 demonstrated interval radiological improvement compared to the admission scan. The study showed ill-defined ground-glass haziness with conglomerate areas of peribronchial dense ground-glass opacification and interstitial thickening involving the right middle lobe and bilateral lower lobes, suggestive of resolving pulmonary hemorrhage with differential considerations of aspiration pneumonitis or pulmonary edema. Few peripheral intraluminal densities were noted within the distal trachea, likely representing retained secretions. Mild mediastinal lymphadenopathy and mild pulmonary arterial hypertension were also noted. Overall, the radiological findings showed significant improvement compared to the previous HRCT chest.

At present, autoimmune workup has not demonstrated definitive evidence of ANCA-associated vasculitis or anti-GBM disease. However, the combination of diffuse alveolar hemorrhage, positive ANA, significant proteinuria, and acute-on-chronic kidney dysfunction continues to raise concern for an underlying autoimmune pulmonary-renal process warranting further outpatient evaluation and follow-up.

Nephrology consultation was again obtained because of the worsening renal function. Review of previous records revealed a baseline creatinine of approximately 1.5 mg/dL in 2025 with gradual progression to 1.8 mg/dL by December 2025 and 2.5

For post-hospital care at home, call Max@Home at 8744 888 888 (24x7 helpline). Services include Critical Care@home, Nursing Healthcare Attendant, Physio-therapy, X-Ray, Sample Collection, Medicine Delivery, Medical Equipment and more.

IN CASE OF ANY EMERGENCY PLEASE CONTACT 0172-5212000.

"This is a computer generated document and does not require any signature"

mg/dL on admission. Urine albumin-creatinine ratio demonstrated sub-nephrotic range proteinuria, while urine microscopy did not reveal significant hematuria. Based on the clinical profile and nephrology assessment, the renal dysfunction was considered more consistent with chronic kidney disease rather than active glomerulonephritis, and renal biopsy was not considered indicated.

Over the course of hospitalization, hemoptysis gradually settled and ultimately resolved. Serial clinical assessment demonstrated progressive improvement in respiratory symptoms. A repeat HRCT chest showed a slight reduction in the extent of bilateral alveolar opacities, consistent with resolving alveolar hemorrhage.

With resolution of active bleeding, improvement in radiological findings, stable respiratory status, and no evidence of active pulmonary-renal vasculitis, the patient remained clinically stable and was transferred to the ward on 03/06/2026 for further care.

Significant Investigations

HRCT Chest

Bilateral perihilar and basal ground-glass opacities suggestive of diffuse alveolar hemorrhage.

Autoimmune Workup

ANA by immunofluorescence: Positive

ENA Profile: Negative

Anti-GBM Antibody: Negative

Renal Evaluation

Baseline creatinine approximately 1.5 mg/dL during 2025

Increased to approximately 1.8 mg/dL in December 2025

Admission creatinine approximately 2.5 mg/dL

Urine ACR: Sub-nephrotic range proteinuria

No RBCs on urine microscopy

Microbiology

Sputum multiplex PCR: Negative

Sputum Gram stain and culture: Negative

For post-hospital care at home, call Max@Home at 8744 888 888 (24x7 helpline). Services include Critical Care@home, Nursing Healthcare Attendant, Physio-therapy, X-Ray, Sample Collection, Medicine Delivery, Medical Equipment and more.

IN CASE OF ANY EMERGENCY PLEASE CONTACT 0172-5212000.

"This is a computer generated document and does not require any signature"

Follow-up Imaging

Repeat HRCT chest showed slight decrease in bilateral alveolar opacities.

Cross-Consultations

Nephrology

Evaluated for pulmonary-renal syndrome.
Urine ACR showed sub-nephrotic proteinuria.
No urinary RBCs.
Renal biopsy not advised.
Impression favored chronic kidney disease rather than active glomerulonephritis.

Pending Results

No major pending investigations reported at the time of discharge.

Condition on Discharge:

Hemodynamically stable
Hemoptysis resolved
Respiratory status stable
No active bleeding
Renal function stable compared to admission
Shifted out of ICU and clinically improving

Functional Status at Discharge

Conscious and oriented
Ambulatory as tolerated
Accepting oral diet
Independent in basic activities with family assistance as required

Medications on Discharge:

Discharge Medications

Tab Ceftum (Cefuroxime) 500 mg – twice daily – 5 days

Cap Happi-D (Rabeprazole/Domperidone) 20 mg/30 mg – once daily empty stomach (7 AM)

For post-hospital care at home, call Max@Home at 8744 888 888 (24x7 helpline). Services include Critical Care@home, Nursing Healthcare Attendant, Physio-therapy, X-Ray, Sample Collection, Medicine Delivery, Medical Equipment and more.

IN CASE OF ANY EMERGENCY PLEASE CONTACT 0172-5212000.

"This is a computer generated document and does not require any signature"

Tab Wysolone (Prednisolone) – tapering dose:
40 mg once daily (10 AM after breakfast) – 5 days →
30 mg once daily – 5 days →
20 mg once daily – 5 days →
10 mg once daily – 5 days → stop

Tab Moxovas (Moxonidine) 0.2 mg – per oral once daily

Tab Metoprolol (Metoprolol) 25 mg – twice daily

Tab Montek-LC (Montelukast/Levocetirizine) – once daily (7 PM)

Syp Lupituss SF (Levocloperastine 20 mg/5 ml) 5 ml – thrice daily

Tab Silicar (Cilostazol) 100 mg – per oral twice daily

Tab Alex Lozenges SF (Dextromethorphan 5 mg) – SOS

Tab Rosuvas (Rosuvastatin) 40 mg – per oral HS (9 PM)

Syp Cremaffin Plus (Liquid Paraffin/Milk of Magnesia/Sodium Picosulfate) 15 ml – HS (10 PM)
(If loose stools → stop)

Tab Bempesta EZ (180/10)(Bempedoic acid and Ezetimibe) once/night

Tab Ecosprin 75 mg po once/night (hold in case of blood in sputum and review immediately to OPD)

Nebulization

Neb Combihale-FB (Formoterol/Budesonide) 1 mg – twice daily (7 AM / 7 PM)

Neb Glycoquick (Glycopyrronium 25 mcg) – twice daily (10 AM / 10 PM)

Neb Levolin (Levosalmolol 1.25 mg) – SOS

When to obtain urgent care:

- Seek immediate medical attention in case of recurrent hemoptysis, worsening breathlessness, fever, reduced urine output, chest pain, altered sensorium, or any other clinical deterioration.

Advice on Discharge:

For post-hospital care at home, call Max@Home at 8744 888 888 (24x7 helpline). Services include Critical Care@home, Nursing Healthcare Attendant, Physio-therapy, X-Ray, Sample Collection, Medicine Delivery, Medical Equipment and more.

IN CASE OF ANY EMERGENCY PLEASE CONTACT 0172-5212000.

"This is a computer generated document and does not require any signature"

SSN No. : 150644284 Patient Name : Rajinder Nath Sharma

-The patient has been advised bed rest for 10 days

Follow up Advice:

Follow up in Pulmonology OPD room no 22 after 5 days.

- Follow up in Nephrology OPD
 - Follow up with Cardiology regarding resumption of anticoagulation and antiplatelet therapy.
- Kindly get CBC, RFT, LFT, CXR before coming to OPD.

/es/ Harpal Singh
Associate Director- Critical Care

Signed: 05 JUN, 2026 11:34

/es/ Harpal Singh
Associate Director- Critical Care
Cosigned: 05 JUN, 2026 11:34
for Pulmonology Deepak Bhasin Team

Entered Date : 02 JUN, 2026 11:15

Prepared By: Harpal Singh

For post-hospital care at home, call Max@Home at 8744 888 888 (24x7 helpline). Services include Critical Care@home, Nursing Healthcare Attendant, Physio-therapy, X-Ray, Sample Collection, Medicine Delivery, Medical Equipment and more.

IN CASE OF ANY EMERGENCY PLEASE CONTACT 0172-5212000.

"This is a computer generated document and does not require any signature"